

OUTLINE OF EVIDENCE – MR CORY LEA SHEPHERD (APPELLANT)

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

I will give evidence of the following. Paragraph references are to the notice to admit facts served 28 August 2026 (the Notice). Topics 2 to 8 each take the document, what it said, and what the department and the console could and could not do afterwards. The documents and dates are facts admitted on 8 September 2026; my evidence is what each was at the console and its effect on me.

- The role, and what turns on the console.** I commenced at Logan Hospital Switchboard on 25 March 2019 as a casual, became permanent from 3 March 2021 and full time from 16 October 2023, a continuous shift worker. The Switchboard is the hospital's communications hub: in a Code Blue or MET call it activates and pages the response team, so an error or delay at the console is an error or delay in the emergency response. I will explain what each code is and what turns on the responding team being reached in time. Between 15 and 50 emergency codes go out in 24 hours, and 200 to 300 calls a shift, many from people distressed, frightened or in crisis; complaints from the public, clinicians and staff arrive at the console first, and those my manager did not action came back to me. The standing condition of the role: from the moment I logged onto the board I was held responsible for what went wrong anywhere in that system, while the means of correcting it, the directory, the on-call roster, my own roster and my pay, were held by others. [Notice ¶¶ 1–38]
- 18 July 2023 to 9 May 2024 – the directory and the after-hours arrangement.** The documents: Ms Stibbard's email to the Switchboard of 18 July 2023, which states she would be "removing everyone's access to the database", that a request made "after hours" would "have to wait until either Chloe or myself are back", and that she had "removed the Contact & Number Changes book out of the room"; Ms Taylor's email of 15 April 2024 at 12:39 pm placing herself and Ms Stibbard on after-hours call "effective from today"; and the further directives of 19 April and 9 May 2024. What that meant at the console: from 18 July 2023 an operator taking a call to a wrong or missing number could not correct it, so it stayed wrong for the next caller until someone else acted; from 15 April 2024 departments and clinicians ringing during my shifts with pager and phone updates asked for Ms Taylor, and on many occasions neither they nor I could have it resolved that shift. I was not consulted before any of those changes. [¶¶ 39–55, 111–113, 162–166, 273]
- 2 to 20 May 2024 – where the calls went.** The documents: the MASPER Registrar's two "Switchboard issues" emails to Ms Taylor of 3 May 2024 at 3:06 pm and 8 May at 5:28 pm, listing nine occasions on which calls reached the wrong team, including one recorded as "MET call team called x5290 asking where MET call was located"; Ms Taylor's reply of 9 May at 9:20 am, five days and eighteen hours after the first, asking Dr Wong for her business hours because they were "not provided on the rosters" and her phone "switched off"; her email to the Switchboard that day at 10:15 am of "many ongoing issues" about calls "transferred to the wrong medical teams"; the Integrated Respiratory Service's emails of 15 and 20 May that "we can not help patients or other clinical staff"; and Ms Taylor's reply to me of 20 May at 4:30 pm that the matter had been discussed "this morning". What followed: the Respondent's List of Documents records no such discussion, and nothing sent to the Service or to Switchboard staff; the Service had written twice across five days with the position unchanged. [¶¶ 56–69, 89–110]
- 15 to 21 May 2024 – the hours, the retraction, and the answer.** The documents: my email of 15 May 2024 at 1:15 pm to Ms Taylor and the Switchboard asking that directives and changes "is made in consultation" and asking her hours; Ms Reese's reply that evening at 6:23 pm, marked High importance, asking me to retract it, which I did; Ms Taylor's email to the whole department two days later, on 17 May at 9:30 am, stating "Otherwise my hours are from 06:30-14:30"; and Ms Reese's reply of 21 May asking which "particular changes and/or directives" I was concerned about. What that meant: the information I had asked for was sent to everyone by the manager two days after I was asked to withdraw the question; until then no one at the console knew when she could be reached. [¶¶ 70–83]
- The roster.** The documents: my six draft rosters and the proposal that a .7 employee could anticipate 70% of a full-time wage; Ms Reese's email of 26 April 2024 that Ms Taylor "was working to fix this error"; her acknowledgement of "a few rostering errors made by Chloe with regards to Cory's line in past rosters"; her email to HR of 10 May 2024 stating "Cory's roster will not be considered"; and the Consultation Paper of November 2024 proposing a "more equitable roster". What that meant: late shifts followed by early shifts, and nights, days off then nights, continued on my line across roster periods; I was not told my proposal would not be considered, and learned of that email only from disclosure in July 2025. [¶¶ 211–223, 167–181]
- The pay.** The documents and what each directed: on 8 April 2024 I asked Ms Taylor to review my pay; on 9 April she directed me to MyHR payroll enquiries; on 3 May Payroll wrote to her, copying me, identifying four fortnights and asking her to "submit an AVAC to correct these shifts for each fortnight"; on 13 May Payroll told me to "speak to your Line Manager"; on 21 May she was "waiting payroll confirmation"; on 28 May I was asked to sign a "Validation of claims older than 3 months". The MyHR submissions report records Ms Taylor as the initiator of each of the five AVACs in the period and me as the initiator of none, and neither AVAC PRN 15397775 nor 15605601 appears in it. The public holidays I was not rostered between February and April 2024 and the difference to my pay. The February 2024 special pandemic leave application: two declines by Ms Taylor and approval on the same material. What that meant: the only person who could make the correction was the one the process kept sending it back to; it was not made over those months. [¶¶ 114–142, 182–210]
- 17 and 18 March 2024, and the fatigue request.** The documents: the roster for those two shifts, finishing at 23:00 and starting at 06:00, and my travel each way; the 2024 Emergency Code Register, recording for the evening of 17 March a MET call and, four minutes later, a Code Blue to the same bed, then a further MET call, and for 18 March, a Monday day shift, six entries between 06:00 and 14:00 including a Code Grey, two MET calls and two neonatal MET calls; Ms Forrest's email of 7 July 2026 that the 8-hour agreement applied only to a staff-initiated swap, which these were not; my request of 8 April 2024; Ms Taylor's reply of 9 April that she had escalated it to HR "to confirm policies around this"; no response until 1 May, when the payment was refused "after a consultation with payroll and Human Resources", so that 19 March came from my own leave; the toolkit at clause 10.4.1 sent to Ms Taylor with no response; and Ms Reese sending the fatigue guideline to HR on 20 May 2024. What the employer says of its records: no fatigue risk assessment was made and no FRMS register kept, those records "do not exist", FRMS training applied only to clinical staff, and fatigue assessment at the Switchboard began only after 30 June 2024. Review Decision 69983 finds "the rostering of these two shifts amounted to unreasonable management action". [¶¶ 224–268]
- June 2023 to June 2024 – what I raised, and what followed it.** The documents: the Communication Book entry of 6 June 2023, a reminder about the monthly update of doctors' contact numbers, and its removal; Ms Taylor's email to the Switchboard that morning that the book was "not used to simply put your point across or make any indirect" criticism; my email of 16 June 2023 recording no consultation on major changes; the concerns I raised on 7 August 2023; my complaint of 13 May 2024, later determined to be a public interest disclosure (content not set out); and my endorsement as a workplace delegate. What followed: on Metro South's own statements, no safety assessment, investigation or change of operating procedure followed any complaint I made about misdirected calls, directory accuracy or fatigue before 30 June 2024, which is what Dr Hawes recorded on 1 July 2024 as "they don't listen to his complaints". What I said is in my emails; I never said anything hostile about Ms Taylor or any other member of staff, and made no Communication Book entry criticising anyone. [¶¶ 143–161, 263–282]
- My health before 18 June 2024.** I had never been diagnosed with or treated for depression. My general practitioner's record of 16 November 2023 notes poor sleep with shift work, that I could not do shifts without a good sleep, no psychological illness such as depression or psychosis, and mood good; melatonin and temazepam were prescribed that day, and after late shifts I would sleep in the car before driving home. The ADHD and anxiety noted in my history in October 2022, and the psychiatrist referral to Dr Amini renewed on 16 May 2024, the day after the request that I retract my email, with melatonin the only medication then current, and that no appointment could be obtained. [General-practice records disclosed by the Respondent; ¶¶ 74–77]
- Onset, diagnosis, and afterwards.** That I last worked a shift on 3 June 2024; that on days in May 2024 and the first week of June 2024 I drove to work, could not bring myself to enter the building, and called in sick from the car park. My symptoms from about 18 June 2024 and their effect on sleep, concentration, self-care and relationships and on my capacity for work, from being certified unfit on 1 July 2024 to the restrictions in the Checklist of 3 July 2026. Attending Dr Slawinski on 28 June 2024, whose note records "stress at work" and "upset by people not following rules", and Dr Hawes on 1 July 2024, whose note records "work stress", that "they withhold pay at times, no overtime-not processed, manipulate his roster- so he works later then earlier". Dr Hawes's certificates and his psychiatrist referral. My first consultation with Dr Krishnaiah on 24 October 2024, when I was told I had major depressive disorder with anxiety state, confirmed by the practice in writing that day, fluoxetine increased and quetiapine added at night; his report of 13 February 2025; and my treatment since. The matters at paragraphs 2 to 8 above, and their effect on me as each occurred and accumulated, are what I reported to each doctor, as their records show; the events those records name are facts admitted on 8 September 2026 (Tabs M1 to M4). Afterwards: meetings scheduled while on certified leave; my employment ended under abandonment provisions in October 2024 while I held current certificates, and my later reinstatement; the months without pay after the injury; my return on reduced hours; and that my Queensland Health email was restricted, so I cannot produce my work emails.

This outline states the topics on which I will give oral evidence. It is not a statement of evidence and is not verified.